



HOUSING FOR HEALTH

Orange County — A CalAIM Housing & Health Nonprofit

AI-Driven Growth Blueprint

Prepared by Technijian | 2026-05-26
Irvine, California | housingforhealthca.org

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01 Executive Summary

2022

501(c)(3) Since

2,412

Members Served in 2024

92%

Kept Permanent Housing

39%

Increased Income (Day
Hab)

Housing for Health is a CalAIM provider: it delivers Enhanced Care Management and the housing Community Supports — navigation, deposits, tenancy and sustaining services, day habilitation, and short-term post-hospitalization housing — to Medi-Cal members experiencing or at risk of homelessness in Orange County, and it bills the Medi-Cal managed care plan for that work. By its own reporting it served 2,412 members in 2024, with 92% of tenancy clients keeping permanent housing. It is also building toward a larger role: a platform that helps other front-line providers deliver the same services to the same standard.

For an organization like this, growth means two things — capturing the funding it has already earned, and serving more people. Both are gated by the same operational reality. Every Medi-Cal dollar flows through a managed-care-plan contract and is released only on a clean claim, and plans are widely reported to be slow to pay community organizations on anything that is not. So the highest-value move is not chasing new money; it is collecting cleanly and completely on the services already being delivered, and freeing care managers to serve more people.

This blueprint is a focused, account-based program with three moves: capture revenue (clean-claim and documentation automation that recovers what is owed and speeds payment), serve more people (faster intake and caseload optimization so the same team reaches more members), and win and comply (the outcomes reporting that wins contract renewals and grants, plus the HIPAA and 42 CFR Part 2 compliance the work requires). Every part of it that touches client information is HIPAA-safe by design.

The Core Opportunity

Capture, don't chase. With CalAIM startup subsidies ending and grants tightening, the highest-return move is collecting cleanly and completely on services Housing for Health is already entitled to bill — clean claims, recovered denials, faster payment.

Serve more people. Caseload economics are set by the Medi-Cal rate; optimizing caseloads and speeding intake lets the same team reach more members — the mission and the margin move together.

AI must be HIPAA-safe — and that is the point. Any system touching member information runs under a Business Associate Agreement on private, enterprise endpoints, with no client data sent to public AI tools and a person reviewing every decision. Doing AI the compliant way is what a Medi-Cal-funded organization requires.

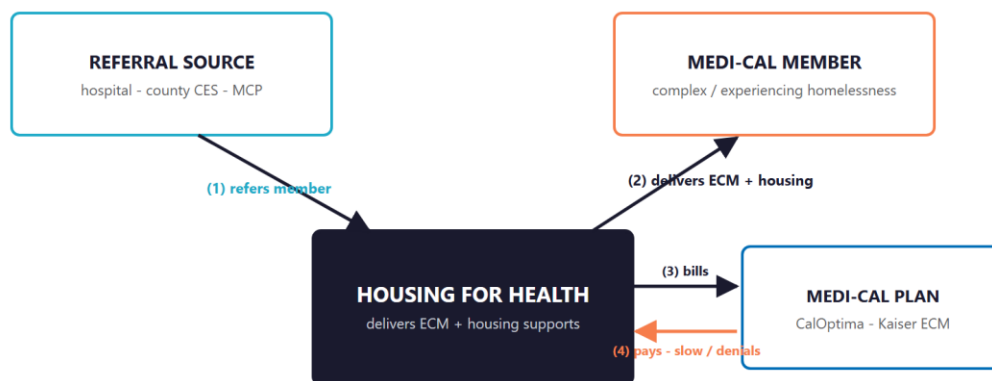
A note on figures: Housing for Health's internal numbers (current Medi-Cal claims volume, denial rate, days to payment, caseload, and current financials) were not available for this draft. Every projection below is labeled estimated and conservative, and calibrates to real numbers after a short discovery call — the specific questions are in Section 15.

02 The CalAIM Model — How Housing for Health Is Funded

A growth plan has to start with how the money actually moves, because in CalAIM the flow is specific and the friction is predictable. A referral source — a hospital discharge, the county Coordinated Entry System, or the health plan’s own high-utilizer list — sends an eligible Medi-Cal member to Housing for Health. Housing for Health delivers Enhanced Care Management and the housing Community Supports, then bills the member’s Medi-Cal managed care plan. The plan pays — and that final step is where revenue leaks, through slow payment and denials on anything that is not a clean claim.

The CalAIM Funding Model - How Housing for Health Gets Paid

A referral becomes an enrolled member; HHOC delivers ECM and housing supports, bills the Medi-Cal plan, and gets paid.



Step 4 is where revenue leaks - slow-pay and denials on anything that is not a clean claim. HHOC also supports partner CBOs.

Where Technijian AI plugs in (HIPAA-safe)

clean claims + denial recovery - documentation & encounters - referral intake & eligibility - caseload optimization - audit-readiness

Figure 2.0 — How Housing for Health is funded: a referral becomes a member, services are delivered, the plan is billed, and payment follows

What Gets Delivered, and How It Pays

Service	What It Is	How It Pays (Representative)
Housing Transition Navigation	Assessment, housing search, applications, landlord engagement, move-in	Per-member-per-month (assumes a set caseload)
Housing Tenancy & Sustaining	Eviction prevention, dispute resolution, lease compliance — keep the housing	Per-member-per-month (a tighter caseload)
Housing Deposits	Security and utility deposits, first month's rent, basic setup (up to ~\$5,000)	One-time, against documentation
Short-Term Post-Hospitalization Housing	Interim housing for members leaving a hospital or institution	Per diem
Enhanced Care Management (ECM)	Whole-person care coordination through a single lead care manager	Per-member-per-month

The Caseload Is the Margin

The per-member-per-month rates bake in an assumed caseload. Staff below that ratio and the rate does not cover the cost to serve; staff above it and quality and audit risk climb. Caseload accuracy is, quite literally, the margin.

A mix of payment models — per-member-per-month, per diem, and one-time — means each service has its own billing rules, and each is a place a claim can be denied or delayed if the documentation is not exactly right.

Housing for Health also supports partner organizations (the emerging platform role), so the same discipline that collects cleanly for its own programs becomes something it can extend to a network.

03 The Growth Equation

Housing for Health grows three ways, and each is gated by something operational that AI can address — within a strict HIPAA-safe boundary. This is the spine the rest of the plan hangs on.

Growth Lever	What Gates It Today	The AI Play (HIPAA-safe)
Capture earned revenue	Denials and slow payment on claims that are not perfectly clean; documentation gaps	Clean-claim generation, documentation completeness checks, denial detection and resubmission
Serve more people	A finite care-management team; intake and eligibility are manual; caseloads set the ceiling	Faster referral intake and eligibility screening; caseload optimization to the rate
Win & deepen funding	Renewals and grants require outcomes the team reports by hand; RFPs are easy to miss	Automated outcomes reporting; account intelligence on plans/counties; RFP and grant discovery

Why the Equation Favors Acting Now

The CalAIM startup subsidies that funded program launch have ended, and federal and state homelessness grants are volatile — so collecting cleanly on the services already delivered matters more than ever.

A new billable service, Transitional Rent, became a required offering in 2026 — a fresh line to capture for an organization ready to bill it cleanly.

Capacity-building funding under CalAIM's PATH initiative is designed to pay for exactly this kind of billing, data, and readiness work — so a meaningful part of the program may be grant-offsettable through 2026.

04 Funding & Regulatory Landscape

Two forces shape the next few years, and both reward the organization that runs cleanly and compliantly. The funding environment is shifting from subsidized launch to self-sustaining operations, and the regulatory environment is heavy — which is both the burden AI removes and the constraint AI must respect.

Funding Forces (2024–2026)

Force	What's Happening	Implication for Housing for Health
Subsidies ending	CalAIM's Incentive Payment Program ended; programs must self-sustain on claims	Collecting cleanly on delivered services is now the core of sustainability
Plans pay slowly	Managed care plans must pay clean claims fast — but getting claims clean is where organizations bleed	Clean-claim and documentation automation is the direct fix
New billable line	Transitional Rent became a required Community Support in 2026	A fresh revenue line to capture early and cleanly
Grants tightening	Federal homelessness funding is volatile; state grants are more prescriptive	Lean harder on the reliable Medi-Cal stream; sharpen grant/RFP targeting
Capacity funding	CalAIM PATH funds billing, data, and readiness work through 2026	A meaningful part of this program may be grant-offsettable

The Regulatory Frame — Burden and Constraint

This work runs on protected health information, so a technology partner is judged first on whether it understands the rules. Most of the load is recurring documentation and reporting that AI automates well — but how AI can be used is bounded, and that boundary is the first thing to get right.

Regime	What It Governs	AI Angle
HIPAA	All protected health information on Medi-Cal members	Constraint first (BAA, encryption, access), then reducer (HIPAA-safe documentation and summarization)
42 CFR Part 2	Tighter confidentiality for substance-use records; compliance required by February 2026	Hard constraint — consent and segmentation; these records do not go to general AI models
Medi-Cal billing integrity	Every claim is auditable; unsupported claims invite clawback	Audit-readiness and documentation-to-claim matching — explainable, human-reviewed, never autonomous billing
HMIS	HUD data standards, de-duplication, reporting	Data-quality automation and reconciliation against plan encounter data
Data Exchange Framework	Statewide health-data exchange; smaller organizations phase in by early 2026	Participate to receive hospital discharge feeds — faster, cleaner referral intake
Fair housing	Anti-discrimination in client and landlord matching	Constraint — any matching model is bias-audited for protected classes

The Constraint We Lead With: HIPAA-Safe by Design

Any AI that touches member information runs under a Business Associate Agreement, on private, enterprise endpoints (the BAA-covered versions of the major AI platforms), tenant-isolated, with no client data used to train a model and no member data ever sent to a public AI tool.

A person reviews anything that affects a claim or a member — AI assembles and flags, people decide. That keeps the work explainable and audit-ready and keeps Housing for Health clear of billing-integrity risk.

This is the difference between an off-the-shelf AI tool and a partner that understands Medi-Cal: the compliance posture is the first design decision, not an afterthought.

05 Where the Growth Lives

Housing for Health’s funding comes from a finite, nameable set of payers and partners — which is why this is an account-based program, not a marketing campaign. There is no consumer funnel here; individuals do not fund the work. The next dollar comes from four pools, and the job of AI is to help capture each one cleanly and to free the team to pursue more of them.

Growth Pool	Who / What	How Housing for Health Captures It
Payer contracts (primary)	The Medi-Cal managed care plan(s) — CalOptima in Orange County, plus ECM through Kaiser	Clean billing that proves reliability, and the outcomes data plans require to renew and expand
Captured / recovered revenue	The claims already earned but denied, delayed, or never billed for lack of documentation	Clean-claim, documentation, and denial-recovery automation
Grants & RFPs	State and county housing funds, foundations, and CalAIM capacity funding	RFP and grant discovery plus drafting from a reusable evidence and outcomes library
Provider network (emerging)	Other housing organizations that need the same billing and compliance backbone	Extend the platform and standards — the role Housing for Health is already building toward

Account-Based by Design — and Mission-Aligned

The buyers are a handful of named plans, counties, and funders — so the work is depth and reliability on those relationships, not broad outreach. AI surfaces the right targets and produces the outcomes evidence that wins them.

The member is at the center, not the market. Serving more people and serving them well is what produces the outcomes that renew contracts — the mission and the funding reinforce each other.

The human relationships stay central. AI handles the documentation, the claims, and the reporting so that care managers and leadership can spend their time on members and partners.

06 The Housing for Health Stakeholders

Housing for Health serves people, but it is funded and sustained by a small set of stakeholders — and a growth program has to understand each one. The cards below profile the relationships that matter most, and the map places each by its influence on funding and the priority it deserves.

1 — The Medi-Cal Managed Care Plan

Who	The plan that funds the work — CalOptima in Orange County, with ECM through Kaiser; the single most important relationship.
What They Need	Reliable, clean billing; strong member outcomes; clean compliance — the evidence that the program is worth renewing and expanding.
What Wins Them	Clean claims that do not create rework, and outcomes reporting that proves housing retention and reduced hospital use.
AI Opportunity	Clean-claim and documentation automation makes Housing for Health an easy plan to work with; automated outcomes reporting proves the value.
Technijian Hook	My Dev — clean-claim and documentation automation. My AI — outcomes and impact reporting.

2 — The Medi-Cal Member

Who	The person served — a Medi-Cal member experiencing or at risk of homelessness, often with complex needs.
What They Need	A faster path from referral to housing, and a care manager with the time to help rather than chase paperwork.
What Helps Them	Quicker intake and eligibility, and a care team freed from manual documentation to focus on the member.
AI Opportunity	Faster intake and a care-coordination copilot give care managers more time per member — the mission metric and a billable one.
Technijian Hook	My Dev — referral intake and eligibility. My AI — care-coordination copilot (HIPAA-safe).

3 — The Referral Source

Who	Hospitals and emergency departments, the county Coordinated Entry System, clinics, and the plan's high-utilizer lists.
What They Need	A simple, reliable way to refer a member and know it was received and acted on.
What Helps Them	A clear intake path and, over time, electronic discharge feeds that route members in automatically.
AI Opportunity	Automated intake from referrals and discharge feeds turns a manual handoff into a fast, trackable enrollment.
Technijian Hook	My Dev — referral intake and eligibility screening. My SEO — a referral-partner page so they can find and use Housing for Health.

4 — The Care Manager

Who	The internal team delivering the work — and the constraint on how many members can be served.
Pain Points	Documentation and stipulation chasing eat the day; caseload pressure trades off against quality and against the rate.
What Helps Them	Automation that turns notes into compliant records and flags gaps before a claim, plus caseload balancing to the rate.
AI Opportunity	Documentation automation and caseload optimization free time, protect margin, and reduce burnout.
Technijian Hook	My Dev — documentation and encounter capture. My AI — caseload optimization.

The Supporting Stakeholders

5 — The Partner CBO

Who	Other housing organizations Housing for Health supports — the emerging platform and network role.
What They Need	The billing, compliance, and reporting backbone they lack the scale to build themselves.
What Wins Them	A proven, modern, compliant platform with the outcomes to show it works.

AI Opportunity	The same automation Housing for Health builds for itself becomes a capability it can extend to a network.
Technijian Hook	My AI and My Dev — the platform and standards. My Security — the compliance that de-risks a network.

6 — The Funder / Foundation

Who	Health-system and philanthropic funders that supplement the Medi-Cal revenue.
What They Need	Clear evidence of impact and a credible, well-run organization.
What Wins Them	Outcomes data and a professional public presence — and proposals that arrive on time and on target.
AI Opportunity	Outcomes reporting and grant discovery and drafting turn impact into funded proposals.
Technijian Hook	My AI — outcomes reporting and grant/RFP discovery. My SEO — the credibility of the public presence.

Figure 6.0 maps each stakeholder by influence on funding and engagement priority — showing the anchor relationships to deepen, the pipeline and strategic relationships to build, and the supplemental ones to maintain.

Stakeholder Map - Influence on Funding x Engagement Priority

HHOC's growth depends on a finite, named set of payers and partners - not a broad consumer market.

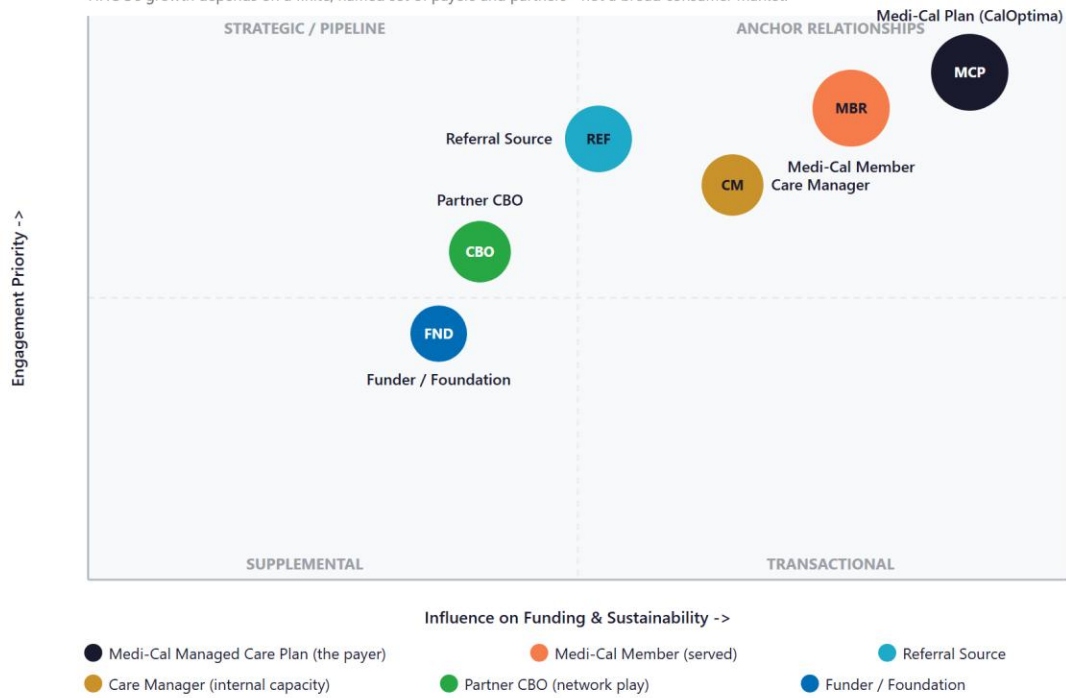


Figure 6.0 — Stakeholder Map: Influence on Funding & Sustainability vs. Engagement Priority

07 Peer & Competitive Landscape

The CalAIM housing space has a clear top tier — organizations that pair scale with real data and outcomes capability — and a large middle that is operations-heavy and digitally behind. Housing for Health cannot out-scale the leaders, but it can out-modernize the operations-heavy peers on data and AI, and that is precisely what wins managed-care contracts and renewals.

Organization	What They Do	Posture vs. Housing for Health
Illumination Foundation	Recuperative care, post-hospital housing, ECM and Community Supports (Orange County)	The tech-forward Orange County benchmark — deep plan-data integration and documented outcomes; the bar to approach
Brilliant Corners	Flexible housing-subsidy administration, navigation, landlord engagement (statewide)	The systems leader at scale — centralized referral, tracking, and reporting; the data benchmark
HealthRIGHT 360	Integrated behavioral health, primary care, and housing (statewide)	Clinically data-mature; large and integrated — a different model, useful as a data reference
PATH	Outreach, shelter, navigation, and ECM (statewide, including Orange County)	Large and established but operations-heavy and not a publicized tech leader — out-modernizable
Mercy House	Full continuum from outreach to permanent housing (Orange County)	Long-tenured regional provider, traditionally behind on technology — a partner more than a threat
Unite Us / findhelp	Closed-loop referral and payment platforms the plans require organizations to use	Infrastructure to interoperate with, not replace — AI sits on top to reduce the multi-platform burden

Scale & Data-Maturity Scorecard

Reduced to the two dimensions that decide managed-care confidence — reach and data/AI maturity — the picture is clear, and it shows the move available to a smaller, well-run organization.

Organization	Scale	Data / Outcomes	AI Posture	Verdict
Brilliant Corners	High	Strong	Forward	Systems leader at scale
Illumination Foundation	Med-High	Strong	Forward	The OC benchmark
PATH	High	Moderate	Behind	Large, operations-heavy
Mercy House	Regional	Light	Behind	Traditional provider
Housing for Health (today)	Small	Emerging	Open	Room to leapfrog on data + AI

Figure 7.0 plots the field. Housing for Health sits in the emerging corner — small and early on data and AI. The realistic move is straight up: out-modernize the operations-heavy peers, approach the benchmark tier on outcomes, and let that win contracts.

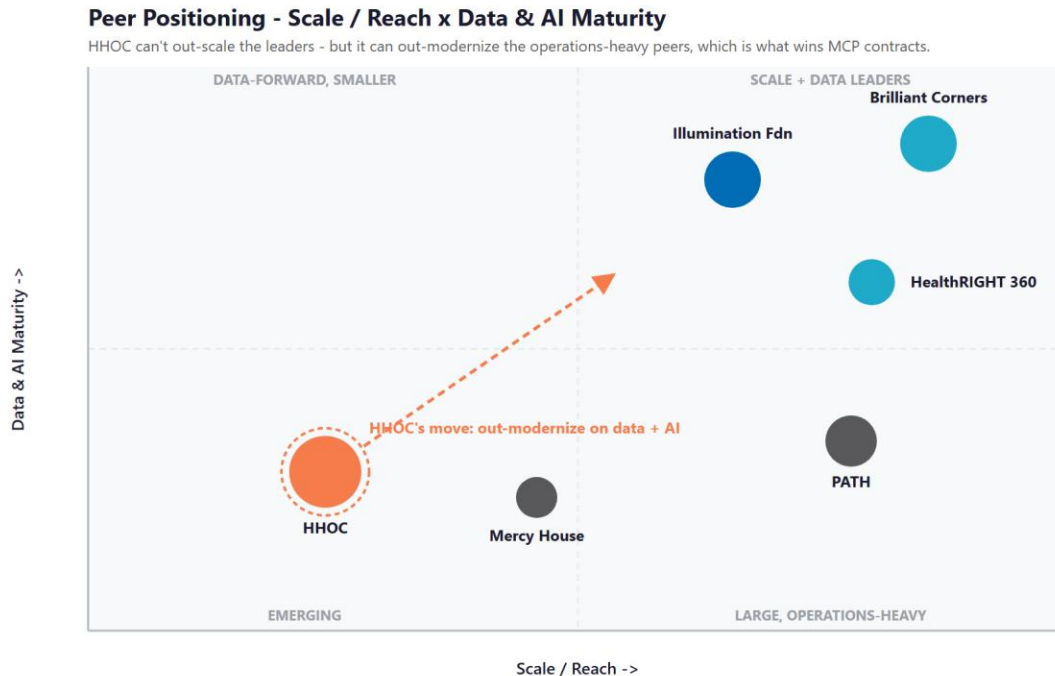


Figure 7.0 — Peer Positioning: Scale / Reach vs. Data & AI Maturity

Where Housing for Health Wins

The field's shared weakness is fragmentation: plans adopted incompatible referral and billing platforms, so care managers juggle multiple portals and manual workarounds. The organization that unifies and automates that work serves more people per dollar.

Outcomes win contracts. The benchmark organizations win and keep managed-care business because they can prove housing retention and reduced hospital use. A smaller organization that reports outcomes just as cleanly competes well above its size.

And the capability compounds: the same automation built for Housing for Health's own programs is exactly what its emerging provider network would pay to use.

08 Brand & Digital Presence Audit

For an organization whose audiences are managed-care plans deciding where to send contracts and partners deciding whether to join a network, the public presence does real work — and today it under-represents a CalAIM provider with genuine outcomes.

Digital Asset	Current State	Gap / Opportunity
Website	Clean, recently rebranded site with a clear mission and an impact page	A strong base — but it speaks more to the public than to the plans and partners who fund the work
Partner page	The “become a partner” path is referenced but not built out	The core of the new network strategy needs a real page that tells plans and CBOs why to engage
Plan / procurement page	No dedicated “for health plans” or procurement section	The buyers are managed-care plans — give them a page that speaks to clean billing, compliance, and outcomes
Outcomes presentation	A few strong metrics, current year only	Outcomes win renewals and grants; presenting them prominently and over time is high-value and low-cost
Content & discoverability	Thin content; limited organic visibility	Referral partners and eligible members should find Housing for Health for “ECM provider Orange County” and similar searches
Data & AI in the workflow	Not evident	Billing, documentation, and reporting appear manual — the gap this blueprint is built to close

The Presence Gap Is the On-Ramp

None of this requires changing the mission — only making a real, proven program look and work as modern as it is.

Fixing the partner and plan pages, presenting outcomes prominently, and improving discoverability are low-cost moves that build credibility with the exact audiences that fund the work.

They are also the natural first ninety days: get the compliance foundation and the public story right, then build the revenue-capture engine on top.

AI Search Reality Check

Here is one part of the gap made concrete. Referral partners — hospital discharge planners, county Coordinated Entry staff, clinic case managers — increasingly ask an AI assistant who can take a Medi-Cal member before they ever open a directory. When a partner asks the question below today, this is the shape of the answer they get:

Prompt: "Who provides CalAIM Enhanced Care Management and housing supports for Medi-Cal members in Orange County?"

TODAY — the AI assistant answers with whichever organizations have the strongest content and third-party signals it can read: it names the larger, more-published CalAIM providers and the county's own pages, and does NOT mention Housing for Health — even though it is a contracted ECM and Community Supports provider with real outcomes. A referrable member can be routed elsewhere at the exact moment a partner is choosing.

AFTER the fix — the same query can surface Housing for Health as a cited option ("Housing for Health Orange County provides CalAIM Enhanced Care Management and housing Community Supports for Medi-Cal members in Orange County..."), with the referral-partner page and the outcomes as the supporting evidence the assistant points to.

(Illustrative of how AI search resolves this query class today, not a screenshot of a specific result; the live result would be captured in the readiness check.)

The Cost of Waiting

The cost of waiting here is not abstract — it is measured in unrecovered Medi-Cal dollars and a closing funding window. Every month of denied or unbilled claims is revenue already earned and then lost; that money does not come back. And the timing is unusually pointed: CalAIM's PATH capacity funding — the exact money that pays for billing, data, and AI-readiness work — winds down at the end of 2026, the CalAIM waiver itself runs through December 31, 2026 with an uncertain successor, and Transitional Rent became a required new billable line in 2026 that a ready organization can capture first. The organizations that modernize their billing and data now capture this window; those that wait pay for the same work later without the grant offset, and leave the recoverable revenue on the table in the meantime.

09 Technijian Capabilities — Proven Builds & Services

This section separates two things, plainly. First are proven builds — systems Technijian has actually built and operates. Then come the productized services Housing for Health would engage. Each is labeled for what it is, and each maps to a specific use case. Everything that touches member information is HIPAA-safe by design.

Proven Builds — Systems Technijian Has Built

AI Document Intelligence for FINRA Broker-Dealers

What Technijian Built: Technijian deployed AI document intelligence that auto-populates complex vendor questionnaires for FINRA-registered broker-dealers, cutting response time from days to minutes with 60–80% less manual review.

How This Applies to Housing for Health: The same approach turns caseworker notes into compliant encounter records and assembles clean claim packets in minutes instead of hours — and drafts grant and RFP responses from a reusable library. Pointed at member data, it runs HIPAA-safe.

Enterprise Knowledge & Memory Systems

What Technijian Built: Technijian implemented enterprise knowledge and memory systems (Weaviate plus Obsidian) that give AI agents persistent, structured recall across an organization's information.

How This Applies to Housing for Health: For Housing for Health this is the basis of a care-coordination copilot that safely summarizes a member's history across systems for the lead care manager, and an institutional memory that survives staff turnover — all under a HIPAA-safe architecture.

Productized Services Housing for Health Would Engage

My Dev — Custom Application Development

The Technijian Service: My Dev is Technijian's custom application development service, built on an AI-native development lifecycle (Claude Code, Figma Make, GitHub, and CI/CD) to ship portals, dashboards, and integrations around an organization's actual process.

How This Applies to Housing for Health: This builds the operating layer: HIPAA-safe clean-claim and billing automation, documentation and encounter capture, caseload and outcomes dashboards, and the referral-intake workflow that feeds it.

My AI — AI Strategy & Implementation (HIPAA-Safe)

The Technijian Service: My AI is Technijian's AI consulting and implementation service — strategy plus hands-on delivery, with security and compliance designed in (HIPAA, SOC 2, and related frameworks), led by a fractional AI advisor.

How This Applies to Housing for Health: For Housing for Health it runs the AI program: the billing, documentation, and compliance automation; caseload and outcomes analytics; and the account intelligence that targets the right plans, counties, and grants.

My Security — HIPAA & 42 CFR Part 2 Safeguards

The Technijian Service: My Security is Technijian's managed security service: the information-security program, monitoring, access controls, and audit trails required to handle protected health information.

How This Applies to Housing for Health: For Housing for Health it delivers the HIPAA safeguards program and the 42 CFR Part 2 readiness due in early 2026 — the compliance foundation that makes AI-assisted work deployable and a low-friction first engagement.

How We Keep AI Affordable — Seven Models, Routed by Task

A fair question from a nonprofit watching every dollar: won't running AI across billing, documentation, outcomes, and account intelligence cost a fortune? Not the way Technijian builds it. We do not wire every task to one expensive model — our platform routes across roughly seven models, spanning three AI vendors and three capability tiers, and sends each sub-task to the cheapest model that can do it well.

Tier	What It Does	Share of Work
Frontier (premium)	The hardest judgment only — final compliance-critical answers, the trickiest denial appeals, deepest reasoning	~5–10%
Workhorse (balanced)	The bulk of drafting and reasoning — documentation, outcomes narratives, grant/RFP drafting, scoring	~30–40%
Lightweight (low-cost)	High-volume mechanical work — classification, extraction, eligibility screening, tagging thousands of records	~50–60%

The result: Housing for Health pays premium-model prices only for the small slice of work that warrants them — typically a 60–80% lower run cost than routing everything to one top-tier model, with no quality loss where it counts. A single grant narrative, for example, is drafted by a low-cost model, tightened and fact-checked by a mid model, and given a final accuracy pass by a frontier model — instead of one premium model doing all three at roughly triple the cost. And it pairs with the HIPAA-safe boundary: the cheap, high-volume models never see member PHI — sensitive data stays inside the private, governed deployment. This is the kind of AI engineering depth a partner brings that wiring everything to one chatbot does not.

10 Understanding AI — A Field Guide for Housing for Health OC Leadership

This section exists to make the rest of this report easy to evaluate. No jargon, no hype — just what AI is, where Housing for Health sits today, how to adopt it without risk, and what comparable organizations are already doing. The goal is that leadership can judge every recommendation that follows on its merits — and see that, for a Medi-Cal-funded nonprofit, the compliant way is the only way we build.

What AI Actually Is — and Isn't

As MIT Sloan puts it, a leader needs to know what AI can and cannot do — not how to build it. In practice, the only distinction that matters for planning is this:

- Automation (workflows): the AI follows a path you define — predictable and low-risk. For example, "turn this visit note into a compliant encounter record and flag what's missing before the claim." This is where almost all near-term value lives.
- Agents: the AI decides the steps itself — more flexible, and it needs human oversight. For example, "watch the claims pipeline and surface the denials worth appealing." This comes later, where it earns its place.

The operating principle (Anthropic's guidance on building AI systems) is to use the simplest thing that works. Housing for Health starts with simple automations that pay off in the first 90 days — clean claims, documentation completeness — and adds more autonomous capability only where the value is proven and a person still reviews every billing decision. That is exactly how the roadmap in this report is sequenced.

Where Housing for Health Sits Today — The AI Maturity Ladder

Most established, well-run nonprofits — including Housing for Health — sit at the first or second rung of a widely-used five-stage AI maturity model (consistent with Gartner and Google Cloud frameworks). The leaders in the CalAIM field are only one or two rungs higher, and the gap closes in months, not years.

Stage	What It Looks Like	Housing for Health Today
1. Foundational	Little or no AI; manual, people-dependent billing, documentation, and reporting	
2. Emerging	A modern, recently-rebranded digital base and real outcomes, but AI is not yet woven into billing, care delivery, or reporting	◀ You are here
3. Operational	AI runs specific workflows day-to-day — clean claims, documentation, outcomes reporting — with measured results	
4. Scaled	AI is embedded across capture, service, and compliance with governance and dashboards	
5. Transformational	AI is the default way the organization runs, scales, and supports its provider network	

This report is the plan to reach Operational — AI capturing revenue, freeing care managers, and proving outcomes — within twelve months, on a HIPAA-safe foundation.

Adopting AI Responsibly — Three Risks Every Leader Manages

The U.S. government's NIST AI Risk Management Framework gives leaders a simple mental model — Govern, Map, Measure, Manage. For a Medi-Cal-funded, PHI-handling organization like Housing for Health, three risks matter most, and each has a concrete control:

Risk	What It Means	How Technijian Controls It
Hallucination	AI can state a confident, wrong answer	Human-in-the-loop review on anything that affects a claim or a member — AI assembles and flags, a person approves; billing is never autonomous
Data leakage	Member PHI pasted into public tools can escape	Private, governed AI under a Business Associate Agreement — member records and 42 CFR Part 2 (SUD) data never touch a public model

Risk	What It Means	How Technijian Controls It
Compliance & accountability	Untracked AI tools create audit gaps	Every AI tool inventoried with owner, vendor, and data source — HIPAA/Part 2-ready, led by a CISSP-certified team

What Comparable Organizations Are Already Doing

- Health & housing operations: multi-site CalAIM and community-services organizations are automating documentation and clean-claim assembly to recover Medi-Cal revenue and free care managers for client-facing work.
- Nonprofit funding: mission-driven organizations are drafting grant and RFP responses from a reusable evidence library — responding to more opportunities with the same small team.
- Regulated intake: PHI-handling providers are using AI to screen referrals and discharge feeds against eligibility rules, turning a manual handoff into a fast, trackable enrollment.

These are representative directions of travel across comparable organizations, not guarantees; Housing for Health's own numbers would be confirmed in discovery. Technijian's specific, measured results from prior builds appear in Section 9 (Capabilities) and Section 11 (the AI engine).

A Day in the Life — A Housing for Health Care Manager

Before vs. After AI

TODAY: A care manager spends much of the day on paperwork — writing up encounters by hand, re-keying the same details across the case-management system and the billing portal, chasing the one missing document that will hold up a claim, and re-juggling caseloads as new referrals arrive. The expertise that keeps claims clean lives in a few experienced people's heads.

WITH AI (HIPAA-safe): The visit note becomes a compliant encounter record in minutes, with any missing item flagged before the claim goes out; referrals and discharge feeds are screened for eligibility automatically; caseloads are balanced to the rate. The care manager reviews and approves — and spends the recovered hours on members, not forms. The standard is captured in a system, so it holds as the team grows and survives a new hire.

Why a Partner — vs. Hiring or Doing It Yourself

Path	Reality
DIY tools	Inexpensive, but Housing for Health assembles, secures, and governs everything — and owns the three risks above alone, with PHI in play
Hire in-house	A capable AI leader typically costs \$180K+/year and is scarce — out of reach for a small nonprofit, and one person cannot cover strategy, build, security, and HIPAA governance
Partner (Technijian)	Strategy, build, security, and HIPAA/Part 2 governance in one team at a fraction of a hire — with proven builds and CISSP-led security, and a program designed to fund itself

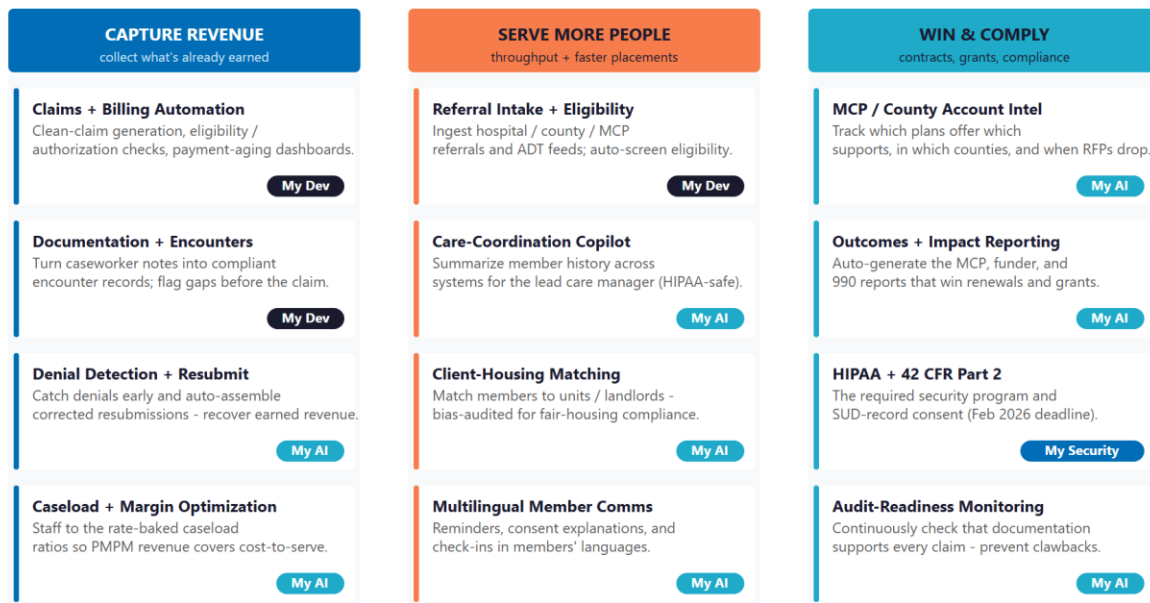
Sources cited in this section: MIT Sloan Management (AI literacy); Anthropic (AI system design); a widely-used five-stage AI maturity model (consistent with Gartner and Google Cloud frameworks); U.S. NIST AI Risk Management Framework. Full references in the Appendix.

11 How AI Helps Housing for Health Serve More & Capture More

The engine runs three motions at once: capture revenue (collect cleanly and completely on services already delivered), serve more people (faster intake and better-balanced caseloads), and win and comply (the outcomes and account intelligence that win funding, on a compliant foundation). Every part that touches member information is HIPAA-safe by design.

Housing for Health AI Engine - Capture More, Serve More, Win & Comply

Collect the Medi-Cal revenue already earned (left), increase throughput (center), and win contracts and stay compliant (right).



* My Dev - My AI - My Security - My SEO. Every AI touching client data is HIPAA-safe (BAA, private endpoints, no PHI to public models, human-in-the-loop); matching is bias-audited; billing

Figure 10.0 — The Engine: Capture Revenue, Serve More People, and Win & Comply

Motion	Play	What It Does	Metric	Service
Capture Revenue	Clean-claim & billing automation	Generate clean claims; verify eligibility and authorization	Days to payment	My Dev
Capture Revenue	Documentation & encounters	Turn notes into compliant records; flag gaps pre-claim	Clean-claim rate	My Dev

Motion	Play	What It Does	Metric	Service
Capture Revenue	Denial detection & resubmission	Catch denials early; assemble corrected resubmissions	Revenue recovered	My AI
Capture Revenue	Caseload & margin optimization	Staff to the rate-baked caseload ratios	Margin per program	My AI
Serve More People	Referral intake & eligibility	Ingest referrals and discharge feeds; auto-screen	Time to enroll	My Dev
Serve More People	Care-coordination copilot	Summarize member history for the lead care manager	Time per member	My AI
Serve More People	Client-housing matching	Match members to units and landlords (bias-audited)	Time to placement	My AI
Win & Comply	Outcomes & impact reporting	Auto-generate plan, funder, and 990 reports	Renewals, grants won	My AI
Win & Comply	Plan / county account intelligence	Track which plans offer what, where, and when RFPs drop	Qualified opportunities	My AI
Win & Comply	HIPAA + 42 CFR Part 2 program	The required security program and SUD-record consent	Compliance posture	My Security
Win & Comply	Audit-readiness monitoring	Continuously check documentation supports every claim	Audit exceptions	My AI

The Honest Boundary

Every AI touching member information is HIPAA-safe — a Business Associate Agreement, private enterprise endpoints, no client data to public models, and no member data used to train a model.

A person decides anything that affects a claim or a member. AI assembles, checks, and flags; care managers and billing staff review and approve. Billing is never autonomous, and any matching model is bias-audited for fair-housing compliance.

That is the point: Housing for Health gets the speed and the captured revenue without taking on compliance or billing-integrity risk it cannot defend.

12 Impact & Service Investment

The model below is built from public and industry benchmarks because Housing for Health's internal numbers were not available for this draft. Every figure is estimated and conservative; the discovery questions in Section 15 replace them with real baselines. The logic holds across a wide range of inputs, because the program is funded largely by revenue it recovers — money already earned but not yet collected.

AI as a Managed Investment — Not a Leap of Faith

The reason most AI spending disappoints is not the technology — it is the lack of measurement. Industry research (McKinsey, State of AI 2025) finds roughly 88% of companies now use AI, but only about 39% see a real profit impact; the difference is discipline, not budget. Technijian runs every engagement with stage-gates: we track adoption, then operational improvement (cleaner claims, recovered revenue), then financial benefit against total cost — and if a pilot does not clear its cost at the gate, we stop and re-scope. Housing for Health carries the upside, not blind risk.

The Entry Offer — The 90-Day AI Revenue-Capture Pilot

Start with one clearly-scoped, fixed-price program — not an open-ended engagement. The pilot stands up the HIPAA-safe compliance foundation and a focused clean-claim and documentation pilot, and proves recovered dollars before any larger build is discussed.

What's Included	Detail	Investment
My Security — HIPAA + 42 CFR Part 2 Safeguards	The required security program and Part 2 readiness due early 2026 — the compliance foundation every AI step depends on	\$800/mo = \$9,600/yr
My AI — Fractional AI Advisor (HIPAA-safe)	AI program lead; clean-claim, documentation, and denial-recovery pilot on a defined set of services; baseline denials and days-to-payment	\$2,000/mo = \$24,000/yr

What's Included	Detail	Investment
My AI — Executive AI Workshop + readiness	Leadership alignment and a HIPAA-safe AI roadmap; fix the partner and health-plan pages	\$5,000 one-time
ENTRY PILOT — 90 DAYS / YEAR 1	Fixed scope, published rates, no large up-front build	~\$38,600 Y1 (much of it PATH-offsettable)

The Pilot Bar — and Our Commitment

Success metric: within 90 days, the pilot recovers or cleanly captures a measurable amount of Medi-Cal revenue that would otherwise have been denied or left unbilled — a number set against the baseline we pull in week one — with the HIPAA/Part 2 foundation in place and a person reviewing every billing decision.

Our commitment: the entry pilot is month-to-month after the initial term — no long lock-in, no obligation to continue if it doesn't hit the metric by day 90, and we will tell you honestly whether it is worth continuing. You carry the upside, not the risk.

Projected Lift (Estimated)

Measure	Estimated Current	With the Program	Direction
Clean-claim rate (first-pass)	Manual, variable	Checked before submission	Fewer denials
Days to payment	Slowed by rework	Faster, cleaner claims	Better cash flow
Denied revenue recovered	Ad hoc	Systematic detection + resubmission	Revenue captured
Members served per care manager	Set by manual workload	Optimized to the rate	Serve more people
Documentation completeness	Variable	Gaps flagged before the claim	Audit-ready
Outcomes reporting	Manual	Automated for plans and funders	Renewals + grants

Measure	Estimated Current	With the Program	Direction
HIPAA / 42 CFR Part 2 readiness	In progress	Programmatic + monitored	Compliant by early 2026

Year-1 ROI Model (Estimated, Conservative Assumptions)

Model Input	Conservative	Target	Aggressive
Captured / recovered Medi-Cal revenue (Y1)*	+\$100,000	+\$250,000	+\$500,000
Technijian Program Investment (Y1)	~\$91,000	~\$91,000	~\$91,000
Modeled ROI Ratio	1.1x	2.7x	5.5x

* Captured revenue = denials recovered, documentation gaps closed before submission, faster and cleaner billing, and caseload throughput. The figure depends on Housing for Health's actual claims volume and denial rate — a key discovery item (Section 15). Not counted in the ratio: more people served, reduced audit risk, and contracts and grants won or renewed. All figures are projected, not guaranteed.

Full Program — Year 1 (~\$91K, the later expansion)

Once the pilot proves the recovered dollars, the program expands to the full revenue-capture engine below — the build that turns clean claims, caseload throughput, outcomes reporting, and account intelligence into routine. This is the later expansion, not the entry ask.

Service	Scope	Monthly	Y1 Total
My AI — Fractional AI Advisor (HIPAA-safe)	AI program lead; billing, documentation, and compliance strategy; plan / RFP account intelligence	\$2,000/mo	\$24,000
My SEO — Referral & Funder Discoverability	Answer-engine and search visibility; “for health plans” and partner pages (not consumer marketing)	\$1,000/mo	\$12,000

Service	Scope	Monthly	Y1 Total
My Security — HIPAA + 42 CFR Part 2 Safeguards	The required security program and Part 2 readiness due early 2026 — the entry point	\$800/mo	\$9,600
My Dev — Custom Build (one-time, phased)	HIPAA-safe clean-claim and billing automation, documentation capture, caseload and outcomes dashboards, referral intake	—	\$40,000
My AI — Executive AI Workshop + readiness (one-time)	Leadership alignment and a HIPAA-safe AI roadmap	—	\$5,000
YEAR-1 TOTAL INVESTMENT	Recurring \$3,800/mo + builds		~\$91,000

The Math That Matters

The program is designed to fund itself: recovering even a modest share of denied or uncaptured Medi-Cal claims, and billing more cleanly going forward, covers the investment — this is money already earned, not new money chased.

A meaningful part of the build qualifies as the billing, data, and readiness work that CalAIM's PATH capacity funding covers through 2026 — so the out-of-pocket cost may be materially lower.

The program is phased — start with the compliance foundation and revenue capture, prove the recovered dollars, then scale outcomes reporting and account intelligence — which lowers the entry point and de-risks the spend.

13 Implementation Roadmap

The roadmap runs on a 90 / 180 / 365-day cadence suited to a compliance-sensitive organization: get the HIPAA-safe foundation right, then turn revenue capture on, then scale outcomes and the funding pipeline. Recovered revenue and faster, cleaner billing are visible inside the first six months; the bigger reporting and account-intelligence work is given realistic runway.

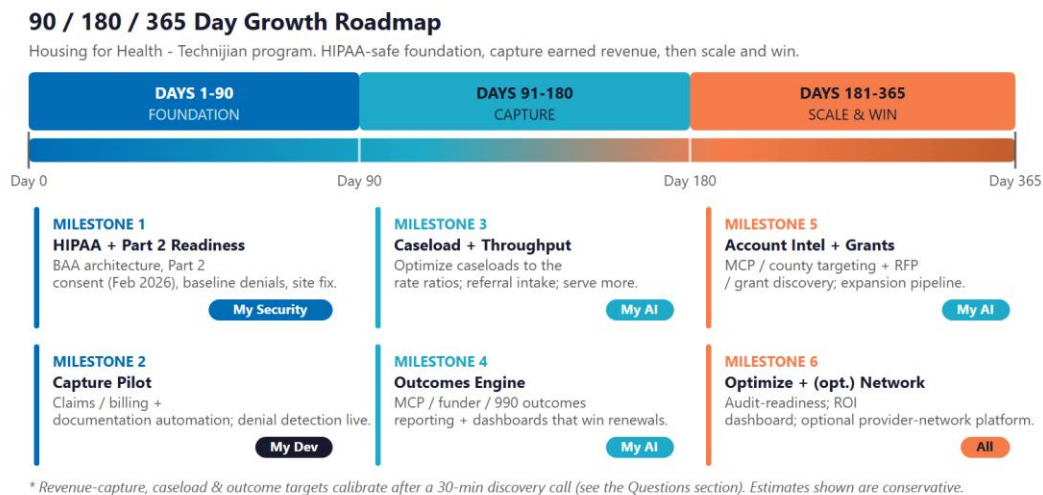


Figure 12.0 — Housing for Health Growth Program: 90 / 180 / 365-Day Roadmap

Phase 1 — Foundation (Days 1–90)

Get the compliance base right and fix the public front door.

Milestone	Deliverables
1.1 — HIPAA & Part 2 Readiness	Stand up the Business-Associate-Agreement architecture and the 42 CFR Part 2 consent handling due in early 2026. Baseline the current denial rate and days to payment. Fix the partner and plan pages.
1.2 — Capture Pilot	Pilot clean-claim and documentation automation with denial detection on a defined set of services — and start recovering revenue that is already earned.

Phase 2 — Capture (Days 91–180)

Turn revenue capture and caseload throughput into routine.

Milestone	Deliverables
2.1 — Caseload & Throughput	Optimize caseloads to the rate-baked ratios and automate referral intake and eligibility so the same team serves more members.
2.2 — Outcomes Engine	Stand up automated outcomes and impact reporting for the managed-care plan, funders, and the annual filing — the evidence that renews contracts.

Phase 3 — Scale & Win (Days 181–365)

Scale the funding pipeline and compound what is working.

Milestone	Deliverables
3.1 — Account Intelligence & Grants	Turn on plan and county account intelligence and RFP and grant discovery to drive renewals and the expansion pipeline.
3.2 — Optimize & (Optional) Network	Bring audit-readiness monitoring into production and deliver the ROI dashboard against the Section 15 baselines — and, optionally, package the platform for the partner-CBO network.

14 Quick Wins — Start This Week

Five actions Housing for Health can take immediately — before any Technijian engagement. Each creates value this week and leads naturally into the larger program.

1 — Build the Partner & Health-Plan Pages

Finish the “become a partner” page and add a short “for health plans” section. These are the audiences that fund the work, and right now they do not have a page that speaks to them — a low-cost fix with outsized credibility return.

2 — Put the Outcomes Front and Center

Housing for Health already reports strong numbers — members served, housing retention, income gains. Present them prominently and keep them current. Outcomes are what win managed-care renewals and grants, so they belong on the front page, not buried.

3 — Baseline the Denials and Days-to-Payment

Pull the current claim denial rate and the average time from service to payment. That single pair of numbers is the starting point for the entire revenue-capture story — and the clearest before-and-after the program can show.

4 — Run a HIPAA & 42 CFR Part 2 Readiness Check

Before any AI touches member data, review how protected health information and substance-use records are handled today against the Part 2 rules due in early 2026. It is a free posture check that de-risks everything that follows — and exactly what a managed-care partner wants to see.

5 — Publish a Referral-Partner Page

Add one clear page telling hospitals, clinics, and county staff how to refer a Medi-Cal member for Enhanced Care Management and housing supports. It makes Housing for Health easy to find and easy to use for the partners who feed the program.

15 Questions to Calibrate This Plan

This blueprint was built from public information. The numbers in Sections 12 and 13 are deliberately conservative estimates — a short discovery call replaces them with Housing for Health’s real baselines and sharpens the entire program. These are the questions that move the model the most:

Topic	What We’d Confirm	Why It Matters
Claims volume	Current Medi-Cal claims volume, denial rate, and average days to payment	Sets the revenue-capture model — the heart of the ROI
Caseload	Current caseloads and care-management staffing against the rate ratios	Calibrates the throughput and margin story
Programs & plans	Which Community Supports are contracted, and with which plans beyond CalOptima and Kaiser	Scopes the billing automation and the account-intelligence targets
Systems	The case-management, HMIS, billing, and closed-loop-referral platforms in use	Defines the build and integration surface
Compliance today	How adverse documentation and Part 2 consent are handled now	Confirms the HIPAA-safe foundation and the Part 2 timeline
Organization model	Whether the focus is direct service or the emerging provider-network platform	Shifts the center of gravity of the AI roadmap
Funding mix	Current funders, any PATH or capacity funding held, and grant priorities	Sizes the grant-offset and the RFP/grant work
Sponsor	Who leads the engagement and how decisions are made	Shapes the rollout and approval path

How to Use This Section

None of these questions block starting — the Quick Wins and the Phase 1 foundation proceed regardless.

A single 30-minute call answers most of them and lets Technijian return a calibrated ROI model and a fixed-scope Statement of Work.

The goal is a plan built on Housing for Health's real numbers — not on our conservative estimates.

16 Questions We Usually Get

The honest answers to the questions Housing for Health leadership is most likely asking right now.

Question	Our Honest Answer
We already have people and partners doing this work. Why add Technijian?	Keep them — they know the mission and the members. We add the layer they don't: HIPAA-safe clean-claim and documentation automation, outcomes reporting, and the account intelligence that wins managed-care contracts. We run the AI program alongside your team and any existing partners, not over them.
Isn't AI mostly hype right now?	A lot of it is. That is why this blueprint starts with simple, proven automations that pay back fast — clean claims and documentation — not autonomous "agents" replacing your care managers. We use the simplest tool that works, measure it, and only expand what earns its place. A person reviews every billing decision.
Is our data — member PHI, substance-use records — safe?	Yes, and this is the first design decision, not an afterthought. Sensitive data never touches a public AI model; we deploy private, governed systems under a Business Associate Agreement, with 42 CFR Part 2 consent handling and human review on anything compliance-bound, led by a CISSP-certified team. The compliance posture comes before any automation.
We're a lean team. Do we have the bandwidth to manage this?	The point is the opposite — to give your team back hours, not add work. Technijian runs the build and the cadence; your involvement is a short monthly strategy session plus reviewing what we draft. The fractional model means no new hire to manage.
What if it doesn't work?	The entry is a fixed-scope 90-day pilot with a defined success metric (Section 12), month-to-month with no long lock-in. If it has not moved the needle on captured revenue by day 90, you are under no obligation to continue — and we will tell you honestly whether it is worth it.

Question	Our Honest Answer
What does it really cost — and can we afford it as a nonprofit?	The program is designed to fund itself by recovering Medi-Cal revenue already earned, and a meaningful part of the build may be offset by CalAIM PATH capacity funding through 2026. The full Year-1 program is profiled in Section 12 at published rates — but you start small, with the pilot, and only scale after the recovered dollars prove the lift.

17 What Happens Next

Housing for Health already has the hard things: a CalAIM provider role, real outcomes, a mission that matters, and the beginnings of a platform that could lift other providers with it. What it has not yet done is put modern data and AI behind the work — to collect cleanly on what it has earned, free its team to serve more people, and prove its impact to the plans and funders who decide its future.

The opportunity is concrete and conservative: capture the Medi-Cal revenue already being left on the table, serve more members with the same team, and win renewals and grants with outcomes reported cleanly — all on a HIPAA-safe foundation, with people in charge of every decision that matters. It is a program designed to pay for itself, with much of the build potentially grant-offsettable.

Recommended Next Steps

Step 1: A 30-minute discovery call to answer the Section 15 questions and confirm program scope.

Step 2: Technijian returns a calibrated ROI model and a fixed-scope Statement of Work within 5 business days.

Step 3: Phase 1 kickoff — the HIPAA-safe foundation, the partner and plan pages, and the first revenue-capture pilot — live inside 30 days of signature.

Ready to capture the funding you've earned — and serve more people?

Contact Ravi Jain, Technijian | RJain@technijian.com | 949.379.8500

technijian.com | Irvine, CA

18 About Technijian

Technijian is an AI-native managed services and technology firm headquartered in Irvine, California, serving small and mid-sized organizations since 2000. We build and operate the AI systems that help mission-driven and regional organizations compete at scale — with security and compliance built in, not bolted on.

Service	What It Delivers for Housing for Health
My AI	HIPAA-safe AI strategy and delivery — clean-claim, documentation, and compliance automation; caseload and outcomes analytics; and the account intelligence that targets the right plans, counties, and grants
My Dev	Custom, AI-native builds — the clean-claim and billing automation, documentation capture, caseload and outcomes dashboards, and referral-intake workflow
My Security	The HIPAA and 42 CFR Part 2 safeguards program, monitoring, and audit trails that make AI-assisted, Medi-Cal-funded work defensible
My SEO	Answer-engine and search visibility plus the partner and health-plan pages that build credibility with the audiences that fund the work

Contact	Detail
Account Executive	Ravi Jain — RJain@technijian.com
Office	18 Technology Dr., Suite 141, Irvine, CA 92618
Phone	(949) 379-8500
Web	technijian.com

A Appendix — Sources

Market and program intelligence gathered via public web research conducted May 2026. Organizational details (founding year, programs, served counts, funders, and footprint) are drawn from public sources and the organization's own materials and should be confirmed with Housing for Health before external use.

1. Housing for Health California — official website: housingforhealthca.org (Home, About, Services, Impact, Contact)
2. ProPublica Nonprofit Explorer & CauseIQ — Form 990 filings (EIN 87-3137292); CMS NPI Registry; CalOptima Health and Kaiser Permanente provider/ECM listings
3. CalAIM — California Health Care Foundation (Housing-Related Community Supports; Billing Better in CalAIM); DHCS (Community Supports Policy Guide; ECM and Community Supports Billing Guidance; Transitional Rent; Incentive Payment Program; Closed-Loop Referral FAQs); NASHP (rate ranges)
4. Regulatory — HHS 42 CFR Part 2 Final Rule (compliance February 2026); HIPAA Privacy/Security; CalHHS Data Exchange Framework; HMIS (HUD) data standards
5. Funding — California HCD HHAP; CalAIM PATH capacity funding; HUD Continuum of Care; health-system and foundation philanthropy
6. Peers — Illumination Foundation; Brilliant Corners; PATH (People Assisting The Homeless); Mercy House; HealthRIGHT 360; Unite Us and findhelp (closed-loop referral platforms)
7. Technijian capabilities & service pricing — My AI, My Dev, My Security, and My SEO; documented Proven Results (AI Document Intelligence for FINRA broker-dealers; Enterprise Knowledge & Memory Systems)
8. AI education layer (Section 10) — MIT Sloan Management Review (AI literacy for executives); Anthropic, "Building Effective Agents" (workflow vs. agent distinction); a widely-used five-stage AI maturity model (consistent with Gartner and Google Cloud AI Adoption frameworks); U.S. NIST AI Risk Management Framework (Govern/Map/Measure/Manage); McKinsey, State of AI 2025 (AI adoption vs. profit-impact discipline)